PRODUCT ASSIGNMENT

Your DOST

Ayush Raj

BITS Goa

LinkedIn Portfolio

OVERVIEW- SOLVING UTILISATION (B2B)

We work in a B2B model where we sign up with corporates to help them with employee well-being. The employees can take counseling sessions online with any expert (though we want them to stick to one expert). They also have access to self-help - including DIY videos, articles, assessments, and structured programs and campaigns on gratitude, smoking cessation, etc.

PROBLEM STATEMENT

External users engage initially but drop off after the first few interactions.

KEY HYPOTHESIS

H1: The "Privacy-Paranoia" Barrier

B2B login (work email / SSO) creates a perceived “surveillance link” between the platform and the employer. Employees fear that their usage, issues, or therapist notes might be visible to HR or managers, so they stop after 1–2 cautious interactions rather than engaging deeply.

H2: The “Vitamin vs. Painkiller” Gap

Users typically arrive in a moment of acute distress (conflict at work, anxiety spike, relationship crisis), expecting fast, targeted relief (“painkiller”). Instead, they face generic, long-horizon wellness content and open libraries (“vitamins”) that require effort and patience, leading to a sharp mismatch in Time-to-Value.

H3: Cognitive Overload & Decision Fatigue

The current experience puts too much decision-making burden on a user who is already mentally exhausted: choose an expert, choose a modality, choose a time, choose a program, choose a video. This unstructured choice architecture produces cognitive overload and decision fatigue, leading many users to drop off instead of taking a clear next step

SECONDARY RESEARCH

- MHFA India cites research indicating that around 75% of employees in India fear stigma and job loss & 67% of employees faced workplace discrimination when disclosing their mental health issues, driven by fear of professional consequences and lack of trust in confidentiality SOURCE
- Industry reports (e.g., Deloitte India, NAMI 2024) indicate that while 80% of employees face stress, ~40% avoid seeking help specifically due to fear of stigma and lack of perceived confidentiality." SOURCE
- In qualitative work on discontinuation of internet-delivered stress-recovery programs, participants reported dropping out when their expectations weren’t met; some described the content as “dull like a textbook,” and not as explanatory or addressing their needs as they expected. SOURCE
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- low early perceived usefulness” and “lack of immediate relevance” are strong predictors of dropout, with users disengaging quickly if they don’t feel the app is helping their specific problem. SOURCE
- UX research on decision paralysis shows that too many choices increase cognitive load, which leads to overwhelm, decision fatigue, and abandonment, especially when all options appear equally weighted. SOURCE

IDENTIFYING THE CORE PROBLEMS

1: Privacy Paranoia

The Fear: "Is HR seeing what I talk about? Will seeking help hurt my promotion?"

What Happens:

- B2B/platform login creates a psychological **"surveillance link"** between user and employer
- Users engage cautiously in the first 1–2 interactions, afraid of exposure
- **Fear** prevents deeper disclosure and repeat sessions
- After initial contact: **User stops re-engaging**

#2: sharp mismatch in Time-to-Value

Users seeking urgent relief for specific stressors ('Painkillers') face a friction-heavy library of generic content ('Vitamins'), causing a critical failure in Time-to-Value.

What Happens:

- Users arrive in **acute distress** (work conflict, anxiety spike, crisis), seeking **fast relief**
- They find a **broad wellness library** ("vitamins") requiring effort and long-term commitment
- Sharp **Time-to-Value mismatch** → perceived irrelevance
- They think: "This won't help my immediate crisis" → **User abandons before booking or completing anything**

SOLUTION SPACE:

#1: Identity Separation for Psychological Safety

Key Idea:

Visually and functionally decouple the user's corporate identity from their platform activity to guarantee psychological safety. Move from "Login" to "Verify & Vanish."

Working:

- Verify: User logs in via Corporate SSO/Email once to validate eligibility.
- Vanish: A clear "Data Firewall" animation plays, explicitly showing the separation of data.
- Alias: The user creates a Pseudonym (e.g., CoolDude) and links a Personal Email for all future notifications, ensuring no "Therapy Reminders" ever appear on work calendars.

What it Solves:

Eliminates the Fear of Surveillance. By creating a distinct "Safe Space" identity, we lower the inhibition threshold, allowing users to be vulnerable and honest without fear of HR repercussions.

#2: The "Pulse" Interface (Immediate Relief)

Key Idea:

Replace the generic content library (Netflix-style) with a "Right Now" Action Interface focused on acute symptom relief.

Working:

- Input: The home screen asks one question: "How are you feeling right now?" with large tappable buttons for acute states (Panic, Anger, Can't Sleep, Low Focus).
- Action: Tapping "Panic" immediately plays a specific 2-minute "Grounding Audio"—no searching, no reading.
- Bridge: Only after the audio ends, the app suggests: "Still feeling stuck? Let an expert help you next steps."

What it Solves:

Fixes the Time-to-Value (TTV) failure. Stressed users have low cognitive bandwidth; this delivers immediate relief (<60 seconds), proving the platform's value instantly before asking for deeper engagement.

#3:"Smart Care Loop" (Retention Engine)

Key Idea:

Eliminate "Dead Ends" by automatically generating a personalized Next Best Action after any key platform interaction (Assessments, Relief Audio, or Sessions).

Working:

- Universal Trigger: The system detects completion of any value-adding activity (e.g., finishing a "Pulse" audio, submitting a Stress Assessment, or ending a chat).
- Contextual Plan: Based on the activity, a dynamic "Care Card" is generated immediately.

1. If the User used "Pulse" (Panic): Plan ⟶ "Check in again in 4 hours" + "Book Expert for root cause."
2. If the User did Assessment: Plan ⟶ “Read interpretation guide" + "Book Discovery Call."

- Commitment & Nudge: The user taps "Accept Plan," setting up a specific notification trigger: "Your expert is waiting for your update."

What it Solves:

Prevents the "Now What?" Drop-off. By instantly bridging the gap between "Passive Consumption" (giving a test or listening to audio) and "Active Commitment" (booking a slot/planning a task), we create a continuous retention loop.

USER FLOW:

ONBOARDING
SSO Login + Firewall animation+ personal gmail + pseudonym(alias creation)

↓

"How are you feeling?" emotion buttons / panic mode

↓

Relief Moment
play grounding audio (other instant relief solutions) + quick check-in

↓

DUAL PATHS
Path A (Expert): Booking → Session
Path B (Self-help): 7-day guided micro-plan

↓

SMART CARE LOOP
Auto-generate next step + user commits + reminder set

↓

Repeat
Notification at user's time → Deep link → Next action → Loop repeats

SUCCESS METRICS:

Metric Name	Calculation Method	Why it Matters?
Day 14 Retention Rate (North Star)	(User active on Day-14 / Users active on Day 0) * 100	This validates if we have successfully converted a curious visitor into a habitual user. Directly addressing churn problem
Alias Creation Rate	(users who create alias / Total SSO logins) * 100	Measures if the "Firewall Animation" & "Incognito Promise" effectively convince users to trust the platform enough to onboard.
Vulnerability Engagement	(Sensitive Interactions (Depression, etc.) \ (Total Interactions (Alias Users))	Validates the hypothesis: "If users feel safe, they will share deeper issues (Burnout/Trauma) instead of just generic ones (Productivity)."
Time-to-First-Relief (TTFR)	(Timestamp (Instant Relief Task complete)) - (Timestamp (App Open))	Measures friction. If this is high, the "Painkiller" solution has failed. We aim for <3 minutes to capture high-stress users.
Loop Closure Rate	(Users completing assigned micro tasks / Total plans generated) * 100	Measures the effectiveness of the "Nudge." A high rate proves users are accepting the "Next Best Action" guidance.
Repeat Expert Booking %	(Users booking appointment with “X” 2nd time / Users booking appointment with “X” 1st Time) * 100	Direct revenue driver. Shows we are building a "Therapeutic Alliance" (Trust) rather than just facilitating one-off transactional calls.
Session Cannibalization	(Free Sessions or material consumption / Paid Sessions) * 100	Ensures the "Free Relief" feature isn't hurting the core business model
Crisis Escalation Miss	% of "Panic" users who exit app immediately without resolution	Safety check. If a user clicks "Panic" and churns instantly, we might be underserving a critical emergency that requires human intervention.

OVERVIEW- MEASURING WELLNESS

Given the subjectivity of wellness, we do not have a way to determine whether users are benefiting from the interventions they engage with (therapy, self-help, awareness, etc.). But to corporates we work with - this is an important aspect to highlight, other than just engagement numbers.

#1 Measuring Wellness (The WHO-5 Delta Model)

Logic:

- We use the WHO-5 Well-Being Index, a globally standardized, clinically validated tool. By assessing users at 15-day intervals
- This eliminates subjectivity and proves the effectiveness of the platform's interventions.

What to Measure:

- Metric: “Organizational Vitality Improvement”
- We measure the aggregate positive shift in employee well-being scores over time.

How to Measure:

Vitality Improvement = (Users with (Score {T15} - Score {T0}) > 0 / Total Active Users * 100)

- Score{T0}= Baseline WHO-5 Score (Day 0)
- Score{T15} = Follow-up WHO-5 Score (Day 15)

What we will show to HR:

HR sees only aggregated changes (no individual responses), reported as organizational Vitality Improvement and a quantified number showing impact.

#2 Immediate Relief (pre/post distress delta)

Logic:

- To measure improvement, we capture the intensity of their distress and their clarity level, without creating friction before the call & treat it as an anchor point
- We measure the "Immediate Delta" (Symptom Reduction) and "Functional Clarity" (Next Steps).

What to Measure:

- We measure the subjective intensity of the user's primary emotion (Anxiety, Confusion, Anger) at the exact moment of entry & after the session.

How to Measure:

Relief Effectiveness% = (Score_Pre - Score_Post) / (Score_Pre) * 100

- Score_Pre = Distress Score before call
- Score_Post = Distress Score after call

What we will show to HR:

Here is the ROI of a 45-minute session: On average, your employees report a 40% reduction in distress intensity immediately after speaking with an expert.

#3 The "Topic Progression" Index (Crisis > Growth)

Key Idea:

- Measuring the shift from "Survival Mode" to "Thriving Mode."
- A user who is unwell consumes content about Acute Symptoms (e.g., "Panic Attacks," "Insomnia," "Depression"). As they get better, their consumption naturally shifts to Maintenance & Growth (e.g., "Leadership," "Focus," "Better Relationships").

Key Idea:

- Metric: “Content Growth Ratio”
- Track the tags of the content/experts the user engages with over time.

Key Idea:

= (Growth/Performance Content Interactions / Total Interactions * 100)

What we will show to HR:

"We saw 40% of users migrate from 'Stress Management' topics to 'Leadership & Focus' topics. Your workforce isn't just coping; they are upskilling."

FEATURE PRIORITIZATION USING RICE FRAMEWORK

FEATURE	REACH	IMPACT	CONFIDENCE	EFFORT	RICE SCORE	WHY IT MATTERS
MEASURING WELLNESS (THE WHO-5 DELTA MODEL)	8000	3	0.85	2	10200	• Works across therapy + self-help + content • Enterprise-grade KPI for procurement and HR • Converts directly to “% vitality improvement” narrative
IMMEDIATE RELIEF (PRE/POST DISTRESS DELTA)	6500	2	0.75	1.5	6500	• Fast time-to-value • Great for sales demos and short-term ROI claims • Complements WHO-5 with “session effectiveness”
THE "TOPIC PROGRESSION" INDEX (CRISIS > GROWTH)	10000	1.5	0.65	3	3250	• Useful as supporting evidence, not primary outcome • Higher analytics complexity for lower perceived value • Harder to explain to buyers

Scale:

- Reach: users/month (absolute)
- Impact: 0.5 low, 1 medium, 2 high, 3 very high
- Confidence: 0–1
- Effort: weeks required to ship

Assumption:

- We currently have ~10,000 monthly active external users (MAU) engaging with at least one intervention (therapy session, self-help module, or content).